

PROVIDER-SPECIFIC AMENDMENT / RIDER

Issued by United Healthcare Insurance Company

Rider ID: UHC-PSA-2025-F4-0021 **Effective Date:** July 1, 2025 **Group Name:** Vertisoft Industries, Inc. **Underlying Policy Reference:** amends_document_id: w14_united_healthcare_detailed_policy_004

This document ("Rider") forms a part of and modifies the Group Health Insurance Policy identified above. It is subject to all terms, conditions, and definitions of the Policy unless expressly amended herein. In the event of any conflict between this Rider and the Policy, the provisions of this Rider shall control. The section being revised is amended_section: Definitions and General Provisions, along with corresponding benefit schedules and coverage terms incorporated by reference.

1. AMENDMENT SCOPE AND EFFECTIVE DATE

1.1 General Purpose

This Provider-Specific Amendment / Rider is entered into between United Healthcare Insurance Company ("Carrier") and the Group named above ("Plan Sponsor"), effective as of the date set forth in the header. It modifies the Definition of "Covered Services," the General Provisions related to cost-sharing, and certain benefit-specific requirements originally set forth in Section 2 (Definitions and General Provisions) of the Policy. The Rider reflects a negotiated network agreement with a designated provider system and incorporates enhanced behavioral health benefits, updated utilization management protocols, and revised out-of-network reimbursement terms.

1.2 Effective Date and Duration

The modifications described herein take effect on the Effective Date shown above and shall remain in force for the remainder of the Policy Year in which the Effective Date falls, and thereafter for each subsequent renewal year unless terminated or superseded by a subsequent amendment. The initial Policy Year to which this Rider applies begins July 1, 2025 and ends June 30, 2026.

1.3 Parties and Enforceability

All provisions of this Rider are binding upon the Carrier, the Plan Sponsor, and each enrolled Member. In accordance with the amended definitions, any reference in the Policy to "Policy" or "Contract" shall be read to include this Rider. Capitalized terms not defined in this Rider retain the meaning given in the underlying Policy (amends_document_id: w14_united_healthcare_detailed_policy_004). The modifications herein are not intended to and shall not be construed to reduce any coverage mandated by state or federal law, including the Affordable Care Act (ACA).

2. MODIFIED COVERAGE TERMS

The following coverage terms replace, in their entirety, the corresponding provisions in the Policy. All cost-sharing amounts are calculated on a Plan Year basis (July 1 - June 30) unless otherwise indicated. Accumulation of deductibles, copayments, and coinsurance toward the out-of-pocket maximum follows the methodologies described in the Policy's original General Provisions, except as explicitly modified below.

2.1 In-Network Medical Benefits

2.1.1 Deductible and Out-of-Pocket Limit

- in_network_deductible: \$1,500

- This amount is the individual calendar-year deductible that must be satisfied before the Plan begins to pay covered medical expenses for services obtained from In-Network Providers. The deductible applies to most medical services except as noted (e.g., certain preventive services covered at 100% without cost-sharing). Family deductible is three times the individual deductible; no single family member will contribute more than the individual deductible toward the family deductible.

- out_of_pocket_max: \$6,500

- Once a Member's total In-Network cost-sharing (deductible, copayments, and coinsurance) reaches \$6,500 in a Plan Year, the Plan pays 100% of the Allowed Amount for covered In-Network services for the remainder of that year. The out-of-pocket maximum includes the in-network deductible, copayments, and coinsurance, but excludes premiums, balance-billed charges, penalties, and the cost of non-covered services.

2.1.2 Coinsurance Structure

After the In-Network deductible is met, the Plan pays 80% of the Allowed Amount for most covered services, and the Member is responsible for 20% coinsurance, until the out-of-pocket maximum is reached. Common exceptions are detailed in the subsections below.

2.2 Out-of-Network Coverage

For services rendered by Non-Participating Providers, the following terms apply:

- out_of_network_coverage: 60% after \$3,000 deductible

- The Member must first satisfy a separate, individual Out-of-Network deductible of \$3,000 per Plan Year. After satisfaction, the Plan will reimburse 60% of the Usual, Customary, and Reasonable (UCR) charge for covered services. The Member is responsible for any difference between the UCR amount and the provider's actual charge (balance billing), which does not count toward the out-of-pocket maximum.

The Out-of-Network deductible and coinsurance do not accumulate toward the In-Network deductible or out-of-pocket maximum, and vice versa. In no event will combined In- and Out-of-Network cost-sharing exceed the federal out-of-pocket limit for qualified health plans.

2.3 Emergency Room Services

- er_copay: \$250

- A flat copayment of \$250 applies to each visit to a hospital emergency department for treatment of an emergency medical condition. The copayment is waived if the Member is admitted to the hospital as an inpatient directly from the emergency department. This copayment is applied after the In-Network deductible (if not yet satisfied) and counts toward the In-Network out-of-pocket maximum. Emergency services received from Out-of-Network providers are reimbursed at the In-Network level, subject to the same cost-sharing, consistent with the ACA's prudent layperson standard.

2.4 Advanced Imaging and Prior Authorization

- mri_prior_auth: Required, 5-day turnaround

- All magnetic resonance imaging (MRI) studies, along with computed tomography (CT) scans, positron emission tomography (PET) scans, and nuclear cardiac imaging, require prior authorization. The Plan shall issue a determination on a non-urgent advanced imaging request within 5 business days of receiving all clinically necessary information. In urgent situations, the determination shall be communicated within 24 hours. Failure to obtain prior authorization for a non-emergent MRI will result in denial of coverage; in such cases, the Member may be held financially responsible for the full billed charge. Authorizations are valid for 45 calendar days from the date of issuance.

For reference, common procedures subject to this requirement include, but are not limited to:

- MRI Lumbar Spine (CPT 72148)
- MRI Brain (CPT 70551)
- CT Abdomen/Pelvis with contrast (CPT 74177)

2.5 Mental Health and Substance Use Disorder Services

- mental_health_visits: 20/year, \$40 copay

- The Plan covers up to 20 outpatient mental health or substance use disorder visits per Plan Year when rendered by an In-Network behavioral health provider. Each visit is subject to a copayment of \$40. The copay applies regardless of whether the In-Network deductible has been met, and it accumulates toward the out-of-pocket maximum. Visits beyond the 20-visit limit are not covered, except as required by the Mental Health Parity and Addiction Equity Act (MHPAEA) for conditions that meet medical necessity criteria for continued treatment; in such cases, additional visits may be authorized at the standard medical coinsurance level (80%/20%).

The 20-visit limit applies separately to mental health therapy and to substance use disorder counseling; however, the copay amount remains \$40 per visit for each service type. In-Network intermediate services (Intensive Outpatient Programs, Partial Hospitalization) are covered at 80%/20% after the deductible, with no visit cap.

2.6 Prescription Drug Benefits

- prescription_tiers: 4 tiers

- The pharmacy benefit is structured in four copayment/coinsurance tiers, as detailed in the table below. These tiers apply to a 30-day supply from an In-Network retail pharmacy; 90-day supplies are available at two-and-a-half times the listed copay. Maintenance medications may be required to be filled through the United Healthcare Pharmacy home delivery program after the third fill.

Tier	Description	Member Cost-Share (In-Network)
Tier 1	Generic drugs (e.g., ICD-10 Z79.899 monitoring)	\$15 copay
Tier 2	Preferred brand-name drugs	\$50 copay
Tier 3	Non-preferred brand-name drugs	\$100 copay
Tier 4	Specialty & high-cost drugs	25% coinsurance (max \$350 per fill)

No deductible applies to Tier 1 and Tier 2 medications; the deductible must be satisfied before the Plan pays for Tier 3 and Tier 4 drugs. The out-of-pocket maximum includes all prescription copayments and coinsurance under this Rider.

2.7 Pre-Existing Condition Limitation

- pre_existing_waiting: None (ACA compliant)

- In compliance with the Affordable Care Act, the Policy does not impose any pre-existing condition exclusion period. Coverage is effective for all covered services from the Member's enrollment date, regardless of any health condition present before enrollment. This Rider reaffirms that no waiting period or limitation related to pre-existing conditions is applied.

2.8 Additional Benefit Modifications

Benefit Category	In-Network Detail	Out-of-Network Detail
Primary Care Visit	\$30 copay, deductible waived	60% after \$3,000 OON deductible
Specialist Office Visit	\$70 copay, deductible waived	60% after \$3,000 OON deductible
Urgent Care Center	\$75 copay, deductible waived	Treated as In-Network if authorized
Inpatient Hospital	80%/20% after deductible	60% after \$3,000 deductible
Outpatient Surgery	80%/20% after deductible	60% after \$3,000 deductible
Laboratory & X-ray	100% after deductible	60% after \$3,000 deductible
Durable Medical Equipment	80%/20% after deductible	60% after \$3,000 deductible
Physical/Occupational Therapy	\$40 copay (60 visits/year combined)	Not covered unless prior authorized

2.9 Accumulation and Financial Consistency

The financial values in this Rider demonstrate internal consistency:

- Once the individual in-network deductible of \$1,500 is satisfied, the member pays 20% coinsurance for most services until reaching the out-of-pocket maximum of \$6,500.

- Example: A member incurring \$40,000 of covered in-network medical claims would first pay the \$1,500 deductible, then 20% of the remaining \$38,500 = \$7,700, but cost-sharing is capped at \$6,500 (including the deductible). Therefore, total maximum out-of-pocket liability is \$6,500.

- Out-of-network cost-sharing runs concurrently; the separate \$3,000 deductible and 60% reimbursement mean significant member exposure, which is separate from the in-network cap.

- All copays (e.g., \$250 ER, \$40 mental health, \$30 PCP) are credited toward the \$6,500 out-of-pocket maximum. Prescription copays/coinsurance also accumulate.

3. SUPERSEDED PROVISIONS

3.1 Identification of Superseded Text

The following provisions of the Policy (amends_document_id: w14_united_healthcare_detailed_policy_004) are hereby deleted and replaced by the terms of this Rider. The affected original Section is amended_section: Definitions and General Provisions, specifically the paragraphs and schedules listed below. Any reference in the Policy to the superseded provisions shall be read as a reference to the corresponding provision of this Rider.

Policy Reference (Original Text)	Superseded By This Rider
Section 2.1 - Definition of "In-Network Deductible" (previously \$2,000)	in_network_deductible: \$1,500
Section 2.2 - Individual Out-of-Pocket Maximum (previously \$7,150)	out_of_pocket_max: \$6,500
Section 2.9 - Emergency Room Copayment (previously \$350)	er_copay: \$250
Section 2.15 - Advanced Imaging Prior Authorization requirement (previously "routine, 7-day turnaround")	mri_prior_auth: Required, 5-day turnaround
Section 2.20 - Mental Health Outpatient Visit Limit & Copay (previously 15 visits, \$60 copay)	mental_health_visits: 20/year, \$40 copay
Section 2.18 - Pre-Existing Condition Limitation (previously "12-month look-back / 6-month waiting" - void under ACA)	pre_existing_waiting: None (ACA compliant)
Section 2.8 - Out-of-Network Reimbursement Rate (previously 50% after \$4,000 deductible)	out_of_network_coverage: 60% after \$3,000 deductible
Section 2.25 - Prescription Drug Tier Structure (previously 3 tiers)	prescription_tiers: 4 tiers

3.2 General Effect of Supersession

All other terms and conditions of the Policy not specifically addressed above remain in full force and effect. The superseding language of this Rider is intended to harmonize with the original intent of the Policy, and any ambiguity shall be resolved in favor of the coverage modifications described in this Rider. No part of this Rider shall be deemed to waive any right, remedy, or defense available to either party under the original Policy or applicable law.

3.3 Member Notice and Acceptance

Enrolled Members will receive a summary of material modifications reflecting the changes set forth in this Rider, as required by the Employee Retirement Income Security Act (ERISA) and applicable state insurance regulations. The Plan Sponsor shall distribute such notice at least 30 days prior to the Effective Date. By paying premium after the Effective Date, the Plan Sponsor and Members are deemed to have accepted the terms of this Amendment.

IN WITNESS WHEREOF, the Carrier has caused this Provider-Specific Amendment / Rider to be executed by its duly authorized representative.

United Healthcare Insurance Company

Signature: _____

Name: _

[illegible]

Date: _